

Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

Answer Choices:

- A. Neonatal respiratory distress syndrome (NRDS)
- B. Tracheoesophageal fistula
- C. Pneumonia
- D. Congenital heart anomaly with right-to-left shunt

Reasoning + Answer:

<think> Reasoning process here : The baby was born after the 32 nd gestational week , which indicates that the baby is not premature . The baby 's symptoms such as tachypnea , subcostal and intercostal retractions , nasal flaring , and cyanosis are indicative of respiratory distress . The cyanosis responding well to oxygen suggests that there is a problem with oxygenation , but not a problem with ventilation . The nasogastric tube was positioned without problems , which suggests that the esophageal opening is not obstructed . The mother 's gestational diabetes is a risk factor for neonatal respiratory distress syndrome (NRDS), but the baby 's symptoms and cyanosis responding to oxygen suggest that NRDS is not the most likely diagnosis . A tracheoesophageal fistula would likely cause feeding difficulties and respiratory distress , but the nasogastric tube was positioned without problems . Pneumonia would likely cause respiratory distress , but the cyanosis responding to oxygen is more consistent with a problem with oxygenation rather than ventilation . </think><answer> D . Congenital heart anomaly with right-to-left shunt </answer>

